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AS AMENDED

By: Rosino of the Senate

Stinson of the House

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

C. The Authority shall award no less than three statewide capitated contracts to provide comprehensive integrated health

1 services including, but not limited to, medical, behavioral health,
2 and pharmacy services and no less than two statewide capitated
3 contracts to provide dental coverage to Medicaid members as
4 specified in Section 4002.3a of this title.

5 D. 1. Except as specified in paragraph 3 of this subsection,
6 at least one capitated contract to provide statewide coverage to
7 Medicaid members shall be awarded to a provider-led entity, as long
8 as the provider-led entity submits a responsive reply to the
9 Authority's request for proposals demonstrating ability to fulfill
10 the contract requirements.

11 2. Effective with the next procurement cycle, and except as
12 specified in paragraph 3 of this subsection, at least one capitated
13 contract to provide statewide coverage to Medicaid members shall be
14 awarded to a provider-owned entity, as long as the provider-owned
15 entity submits a responsive reply to the Authority's request for
16 proposals demonstrating ability to fulfill the contract
17 requirements.

18 3. If no provider-led entity or provider-owned entity submits a
19 responsive reply to the Authority's request for proposals
20 demonstrating ability to fulfill the contract requirements, the
21 Authority shall not be required to contract for statewide coverage
22 with a provider-led entity or provider-owned entity.

23 4. The Authority shall develop a scoring methodology for the
24 request for proposals that affords preferential scoring to provider-

1 led entities and provider-owned entities, as long as the provider-
2 led entity and provider-owned entity otherwise demonstrate an
3 ability to fulfill the contract requirements. The preferential
4 scoring methodology shall include opportunities to award additional
5 points to provider-led entities and provider-owned entities based on
6 certain factors including, but not limited to:

- 7 a. broad provider participation in ownership and
8 governance structure,
 - 9 b. demonstrated experience in care coordination and care
10 management for Medicaid members across a variety of
11 service types including, but not limited to, primary
12 care and behavioral health,
 - 13 c. demonstrated experience in Medicare or Medicaid
14 accountable care organizations or other Medicare or
15 Medicaid alternative payment models, Medicare or
16 Medicaid value-based payment arrangements, or Medicare
17 or Medicaid risk-sharing arrangements including, but
18 not limited to, innovation models of the Center for
19 Medicare and Medicaid Innovation of the Centers for
20 Medicare and Medicaid Services, or value-based payment
21 arrangements or risk-sharing arrangements in the
22 commercial health care market, and
 - 23 d. other relevant factors identified by the Authority.
- 24

1 E. The Authority may select at least one provider-led entity or
2 one provider-owned entity for the urban region if:

3 1. The provider-led entity or provider-owned entity submits a
4 responsive reply to the Authority's request for proposals
5 demonstrating ability to fulfill the contract requirements; and

6 2. The provider-led entity or provider-owned entity
7 demonstrates the ability, and agrees continually, to expand its
8 coverage area throughout the contract term and to develop statewide
9 operational readiness within a time frame set by the Authority but
10 not mandated before five (5) years.

11 F. At the discretion of the Authority, capitated contracts may
12 be extended to ensure there are no gaps in coverage that may result
13 from termination of a capitated contract; provided, the total
14 contracting period for a capitated contract shall not exceed seven
15 (7) years.

16 G. At the end of the contracting period, the Authority shall
17 solicit and award new contracts as provided by this section and
18 Section 4002.3a of this title.

19 H. At the discretion of the Authority, subject to appropriate
20 notice to the Legislature and the Centers for Medicare and Medicaid
21 Services, the Authority may approve a delay in the implementation of
22 one or more capitated contracts to ensure financial and operational
23 readiness.

1 I. Effective with the next procurement cycle, a contracted
2 entity that currently holds a capitated contract with the Authority
3 under the Ensuring Access to Medicaid Act shall be ineligible for a
4 capitated contract award for the subsequent procurement cycle if the
5 contracted entity fails to meet the minimum primary care expense
6 requirement stipulated in subsection O of Section 4002.12 of this
7 title.

8 SECTION 2. AMENDATORY 56 O.S. 2021, Section 4002.12, as
9 last amended by Section 7, Chapter 448, O.S.L. 2024 (56 O.S. Supp.
10 2024, Section 4002.12), is amended to read as follows:

11 Section 4002.12. A. Until July 1, 2027, the Oklahoma Health
12 Care Authority shall establish minimum rates of reimbursement from
13 contracted entities to providers who elect not to enter into value-
14 based payment arrangements under subsection B of this section or
15 other alternative payment agreements for health care items and
16 services furnished by such providers to enrollees of the state
17 Medicaid program. Except as provided by subsection I of this
18 section, until July 1, 2027, such reimbursement rates shall be equal
19 to or greater than:

20 1. For an item or service provided by a participating provider
21 who is in the network of the contracted entity, one hundred percent
22 (100%) of the reimbursement rate for the applicable service in the
23 applicable fee schedule of the Authority; or
24

1 2. For an item or service provided by a non-participating
2 provider or a provider who is not in the network of the contracted
3 entity, ninety percent (90%) of the reimbursement rate for the
4 applicable service in the applicable fee schedule of the Authority
5 as of January 1, 2021.

6 B. A contracted entity shall offer value-based payment
7 arrangements to all providers in its network capable of entering
8 into value-based payment arrangements. Such arrangements shall be
9 optional for the provider but shall be tied to reimbursement
10 incentives when quality metrics are met. The quality measures used
11 by a contracted entity to determine reimbursement amounts to
12 providers in value-based payment arrangements shall align with the
13 quality measures of the Authority for contracted entities.

14 C. Notwithstanding any other provision of this section, the
15 Authority shall comply with payment methodologies required by
16 federal law or regulation for specific types of providers including,
17 but not limited to, Federally Qualified Health Centers, rural health
18 clinics, pharmacies, Indian Health Care Providers and emergency
19 services.

20 D. A contracted entity shall offer all rural health clinics
21 (RHCs) contracts that reimburse RHCs using the methodology in place
22 for each specific RHC prior to January 1, 2023, including any and
23 all annual rate updates. The contracted entity shall comply with
24 all federal program rules and requirements, and the transformed

1 Medicaid delivery system shall not interfere with the program as
2 designed.

3 E. The Oklahoma Health Care Authority shall establish minimum
4 rates of reimbursement from contracted entities to Certified
5 Community Behavioral Health Clinic (CCBHC) providers who elect
6 alternative payment arrangements equal to the prospective payment
7 system rate under the Medicaid State Plan.

8 F. The Authority shall establish an incentive payment under the
9 Supplemental Hospital Offset Payment Program that is determined by
10 value-based outcomes for providers other than hospitals.

11 G. Psychologist reimbursement shall reflect outcomes.
12 Reimbursement shall not be limited to therapy and shall include but
13 not be limited to testing and assessment.

14 H. Coverage for Medicaid ground transportation services by
15 licensed Oklahoma emergency medical services shall be reimbursed at
16 no less than the published Medicaid rates as set by the Authority.
17 All currently published Medicaid Healthcare Common Procedure Coding
18 System (HCPCS) codes paid by the Authority shall continue to be paid
19 by the contracted entity. The contracted entity shall comply with
20 all reimbursement policies established by the Authority for the
21 ambulance providers. Contracted entities shall accept the modifiers
22 established by the Centers for Medicare and Medicaid Services
23 currently in use by Medicare at the time of the transport of a
24 member that is dually eligible for Medicare and Medicaid.

1 I. 1. The rate paid to participating pharmacy providers is
2 independent of subsection A of this section and shall be the same as
3 the fee-for-service rate employed by the Authority for the Medicaid
4 program as stated in the payment methodology in OAC 317:30-5-78,
5 unless the participating pharmacy provider elects to enter into
6 other alternative payment agreements.

7 2. A pharmacy or pharmacist shall receive direct payment or
8 reimbursement from the Authority or contracted entity when providing
9 a health care service to the Medicaid member at a rate no less than
10 that of other health care providers for providing the same service.

11 J. Notwithstanding any other provision of this section,
12 anesthesia shall continue to be reimbursed equal to or greater than
13 the anesthesia fee schedule established by the Authority as of
14 January 1, 2021. Anesthesia providers may also enter into value-
15 based payment arrangements under this section or alternative payment
16 arrangements for services furnished to Medicaid members.

17 K. The Authority shall specify in the requests for proposals a
18 reasonable time frame in which a contracted entity shall have
19 entered into a certain percentage, as determined by the Authority,
20 of value-based contracts with providers.

21 L. Capitation rates established by the Oklahoma Health Care
22 Authority and paid to contracted entities under capitated contracts
23 shall be updated annually and in accordance with 42 C.F.R., Section
24 438.3. Capitation rates shall be approved as actuarially sound as

1 determined by the Centers for Medicare and Medicaid Services in
2 accordance with 42 C.F.R., Section 438.4 and the following:

3 1. Actuarial calculations must include utilization and
4 expenditure assumptions consistent with industry and local
5 standards; and

6 2. Capitation rates shall be risk-adjusted and shall include a
7 portion that is at risk for achievement of quality and outcomes
8 measures.

9 M. The Authority may establish a symmetric risk corridor for
10 contracted entities.

11 N. The Authority shall establish a process for annual recovery
12 of funds from, or assessment of penalties on, contracted entities
13 that do not meet the medical loss ratio standards stipulated in
14 Section 4002.5 of this title.

15 O. 1. The Authority shall, through the financial reporting
16 required under subsection G of Section 4002.12b of this title,
17 determine the percentage of health care expenses by each contracted
18 entity on primary care services.

19 2. Not later than the end of the fourth year of the initial
20 contracting period, each contracted entity shall be currently
21 spending not less than eleven percent (11%) of its total health care
22 expenses on primary care services.

23 3. The Authority shall monitor the primary care spending of
24 each contracted entity and require each contracted entity to

1 maintain the level of spending on primary care services stipulated
2 in paragraph 2 of this subsection.

3 4. If a contracted entity fails to meet the minimum primary
4 care expense requirement stipulated in paragraph 2 of this
5 subsection, the contracted entity shall be ineligible for a
6 capitated contract award for the subsequent procurement cycle as
7 provided by subsection I of Section 4002.3b of this title.

8 SECTION 3. This act shall become effective July 1, 2025.

9 SECTION 4. It being immediately necessary for the preservation
10 of the public peace, health or safety, an emergency is hereby
11 declared to exist, by reason whereof this act shall take effect and
12 be in full force from and after its passage and approval.

13 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
14 February 24, 2025 - DO PASS AS AMENDED
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